THE MECHANIST'S PROTOCOL:
NEURO-CHEMICAL TOXICITIES & BEHAVIORAL MALFUNCTIONS

FOR ADVANCED PHYSIOLOGY INITIATES, MEDICAL MECHANISTS, & NURSING PROFESSIONALS (NCLEX)

E DENIAL CIRCUIT

L IS THE #1 PSYCHOLOGICAL MALFUNCTION

ARY BARRIER: Subject's outtight
l to accept reality of systemic problem
se ecenarios (alcohol, drugs, gembling)

TION PROTOCOL

CTIVE: Point out
ropency
d actions (e.g.,
while consuming
.

CRIEF EXCEPTION

d in profound loss/3 stages (DABOA).
col shifts to structural support, not
ontation.

WERNICKE-KORSAKOFF SYNDROME (WKS)

DUAL-PHASE NEUROLOGICAL DISASTER

WERNICKE'S: ENCEPHALOPATHY (Acute)
KORSAKOFFS: SEVERE PSYCHOSIS (Chronic)
Induced by a critical Vitamin B1 deficit

THE CONFABULATION MECHANISM

Subjects suffer profound amnesia and fabricate intricate stories, genuinely believed as absolute reality

THE REDIRECTION INTERVENTION

DEMENTIA-BASED PSYCHOSIS:
Do not contront the lie; acknowledge feeling and redirect to a safe, foasible activity

PATHOLOGICAL SYMB

DEPENDENCY VS. CODEPEN

DEPENDENCY:
Abuser forces
uthery to make
decisions

CODEPE
Partner c
self-este
performi

BREAKING THE PATHOLOGICAL LOO

Codependent m
iimits, aggressi
and structurally

THE MANIPULATION THRESHOLD

Abuxor coerces partner into
dengerous, harmful, or Illegal acts

THIAMINE (B1) - THE GENESIS GEAR

70% IRREVERSIBLE DAMAGE

WKS is preventable and degradation arrested with Vitamin B1, but neural destruction is often permanent.

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GLUCOSE METABOLISM FAILURE

Without Thiamine, the brain loses ability to metabolize glucose, leading to systemic "brain rot"

ANTABUSE (DISULFIRAM) - THE AVERSIO

BEHAVIORAL AVERSION THERA

Disultiram alters lleer enzymes to
induce violent, catastrophic illness
trace alcohol is ingested

THE 2-WEEK CHRONOMETR

Medication requires TWO WEEKS t
build therapeutic effectiveness an
TWO WEEKS to cafety claer before
alcohol consumption

PROHIBITED REAGENTS

Subjects must avoid: mouthwesh,
colegne, perfome, aftershave, eliair
OTC liquid medicines, losect regulla
vanilla extract, and hand sanitizers